

Gaurav Rajendra More

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📄 PROFILE

- Master's in **Biomedical Engineering(Biostatistics)** with expertise in clinical and epidemiological data analysis, applying statistical models, survival analysis and hypothesis testing to support health research.
- Proficient in **R, SAS, Python, and SQL** for data analysis and reporting; experienced with **REDCap, Shiny, Bioconductor**, and **CI/CD workflows** for reproducible, regulatory-compliant outputs.

🎓 EDUCATION

The Ohio State University, Columbus, Ohio Master's in Biomedical Engineering (Biostatistics) <i>Relevant Courses:</i> Clinical Trials & Experimental Design Survival Analysis Generalized Linear Models Longitudinal Data Analysis Epidemiologic Methods Statistical Computing with R & SAS	08/2023 – 05/2025
Bharati Vidyapeeth College of Engineering (Mumbai University), Navi Mumbai B.E Chemical Engineering (Statistics)	08/2019 – 06/2023

🧠 SKILLS

Statistical Programming: R | SAS | SQL | Python

Methods: Logistic Regression | Survival Analysis | Cox Models | Mixed Effects Models | Hypothesis Testing | Power Analysis

Tools & Frameworks: Shiny | ggplot2 | R Markdown | Tableau | Git | REDCap

Clinical Standards: CDISC (SDTM/ADaM) | GxP Standards

Workflow: CI/CD | Agile | GitHub | Linux | Slurm

💼 WORK EXPERIENCE

Graduate Research Assistant, The Ohio State University • Analyzed over 12,000 clinical records using SAS and SQL to study obesity and cardiovascular risk; reduced data processing time by 35% via automated scripts. • Applied logistic regression & Cox models to identify predictors of adverse outcomes ($p < 0.01$) in a diabetes cohort ($n = 1,500$). • Built Shiny dashboards for 3 clinical departments to monitor BMI, HbA1c, and adherence trends. • Performed power analysis ensuring 90% power to detect a 10% BP reduction; co-authored 2 publications and 3 IRB protocols .	08/2023 – 05/2025 Columbus, United States
Research Analyst Co-op, NavaFlex (formerly FlexEnergy LLC) • Developed reproducible pipelines in R Markdown & SAS , reducing report generation time by 50% for a chronic disease study ($n = 5,000$). • Conducted Kaplan-Meier and Cox regression ($HR = 2.3$, 95% CI: 1.8–2.9) to model disease progression risk. • Collaborated in Agile teams and Implemented CI/CD validation scripts with Git , ensuring reproducible, audit-ready results. • Ensured 100% compliance with CDISC SDTM/ADaM standards for regulatory submissions. • Built Tableau dashboards used across 18 sites for biomarker trend monitoring.	02/2024 – 08/2024 Columbus, United States
Research Analyst Intern, Mechmann Engineering • Applied computational biology techniques to analyze biological data from over 1,000 cancer clinical trials using R, SAS to find which treatments worked best for different patients. • Used logistic regression models to recommend better dosing, helping reduce treatment side effects by 30% . • Researched biomarkers in extracellular vesicles to support drug delivery ideas and improve future cancer trial design.	01/2023 – 06/2023 Mumbai, India

📁 PROJECTS

Risk Modeling for Type 2 Diabetes (T2D) Progression, Academic Project • Developed predictive models (logistic regression, LASSO) on integrated clinical and omics data (47,660 features, 234 patients), achieving AUC of 0.91 and Sensitivity of 94% . • Addressed class imbalance using oversampling; performed variable selection and model validation via bootstrapping, ROC, and calibration curves . • Built interactive Shiny dashboards to display biomarker associations and model diagnostics; created reproducible analysis pipeline with R Markdown and Git.	01/2025 – 05/2025
Predicting Emergency Department Outcomes Using Machine Learning, Course-Based Capstone Project • Built ML models using MIMIC-IV-ED data to predict emergency outcomes like hospital admission, discharge, or mortality using early-available features. • Processed both structured data (vitals, demographics) and unstructured data (chief complaints) using NLP and feature engineering . • Achieved strong results using XGBoost and Me-LLaMA , with the best model reaching an AUC-ROC of 0.88 , helping improve triage support and resource planning in emergency care.	09/2024 – 12/2024